

All figures sourced from Government of Alberta open data, Statistics Canada, federal statute, UN treaty, and peer-reviewed research.

Building Up: A Blueprint for the Alberta Accessibility and Inclusion Act

Part One — What Real Accessibility and Inclusion Actually Look Like

Section 1.4 — Transition and Adulthood: PDD, AISH, and the Architecture That Should Catch People

Section 1.3 followed the disabled child through the developmental window and documented Alberta's failure to provide the growth-mode supports the science establishes as time-bound and the economic literature establishes as cost-recovering. Section 1.4 follows the same population into the next life stage. At eighteen, a disabled Albertan crosses two thresholds simultaneously. Family Support for Children with Disabilities (FSCD) eligibility ends. School-system supports end. The adult disability architecture is supposed to catch the child the previous architecture was supposed to support. The principal mechanisms are the Persons with Developmental Disabilities (PDD) program for adults requiring lifelong-support architecture, and the Assured Income for the Severely Handicapped (AISH) program for adults requiring stability-mode income support.

Both programs are currently failing. The failures are documented, sourced, and quantified. They are not the result of insufficient resources, novel demand, or unforeseen population growth. They are the consequence of deliberate program design choices that have made the bridge between childhood and adulthood, for disabled Albertans, a cliff. The same lifecycle-frame failure pattern documented in Section 1.3 recurs at the adulthood threshold, with the same structural error: the system accepts the disability as the basis for eligibility, then deploys the disability as the barrier to navigating eligibility. The contradiction is documented in Alberta's own program data. It is unsustainable as policy and unconscionable as law.

This section examines what the adult architecture should provide, what Alberta currently provides, and what the cumulative failure produces.

The Cliff at Eighteen

A disabled child raised under Alberta's current architecture arrives at the eighteenth birthday in one of two situations. In the first, the child has been receiving FSCD services and school-system supports throughout childhood, and the family has navigated the transition planning that the FSCD program is supposed to provide. In the second, the child has been on the FSCD waitlist for some or all of childhood, has received partial or no FSCD services, and arrives at the eighteenth birthday with the developmental window already foreclosed and no transition plan in place. In both situations, the child is now legally an adult, FSCD eligibility has ended, school supports are ending or about to end, and the adult disability architecture must absorb whatever the previous architecture has produced.

The Government of Alberta's own AISH application guidance directs applicants to *apply for AISH six months before you turn 18*. The directive is reasonable on its face. It assumes that the family or the disabled adult-to-be has the administrative capacity to identify the deadline, understand the application requirements, gather the necessary documentation, navigate the form, secure a physician appointment,

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pay the physician fee for completion of the Part B Medical Report, and submit the complete application package within the window. For the population the program is designed to serve — adults with severe and permanent disabilities, including cognitive disability, intellectual disability, severe mental illness, and the comorbid conditions that frequently accompany them — this directive is the entry point into the structural contradiction the rest of this section examines. The administrative capacity the directive assumes is the administrative capacity the disability the program tests for routinely impairs.

For PDD, the parallel process operates on a separate timeline and through separate criteria. PDD applications are accepted from individuals sixteen years old and up, with the eligibility threshold defined as a developmental disability with onset before age eighteen, an IQ at or below seventy, and significant adaptive skill limitations. The program serves over 13,000 adults with intellectual disabilities. Many adults qualify for both PDD and AISH and apply to both. PDD approval after August 2009 substitutes for the AISH Part B Medical Report, which is the program architecture's one functional integration point: a person already medically established as eligible for one program does not have to be medically re-established as eligible for the other.

Beyond that single integration, the cliff is structural. The two programs operate independently, with separate caseworker systems, separate review processes, separate eligibility timelines, and separate appeal mechanisms. The disabled adult turning eighteen must navigate both, if both apply. The family that supported the child through FSCD must rebuild its understanding of an entirely different administrative architecture for the adult systems. The community organizations that supported the FSCD application have been defunded or under-resourced, as documented in Section 1.3. The transition is, for many families, the moment at which the supports the child had been receiving simply end, and the supports the adult might receive depend on a multi-month, multi-stage application process the family must navigate without the institutional knowledge the previous decade required them to build for a different program entirely.

What PDD Is Supposed to Deliver

The Persons with Developmental Disabilities program is the lifelong-support architecture in Alberta's adult disability system. It is the program that funds the residential support, employment support, and community participation supports that adults with intellectual disabilities require to live, as the program's own website states, *as independently as possible in their community*. Services may be delivered through one of approximately 160 contracted non-profit and for-profit providers, or through Family Managed Services (FMS), an individualized self-directed funding model under which the adult or family hires support workers and manages care directly.

PDD's design, on paper, is appropriate to the lifelong-support mode the population requires. In operation, the program is failing the population it serves and the population on its waitlist, in ways that Inclusion Alberta documented in its January 2025 survey report *Current State: Critical & Urgent: Experiences with Persons with Developmental Disabilities*. The survey collected responses from 540 individuals across 79 of Alberta's 87 provincial electoral districts in October and November 2024. The data establishes the following:

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Of survey respondents waiting for an eligibility decision, the average wait was 144 days, or approximately four and a half months. Of respondents who had recently received an eligibility decision, 39 percent had waited ten months or longer for that decision after applying. Of respondents who had received a positive eligibility decision but had not yet received PDD services, the average wait was 14 months from the date of the eligibility decision. Of respondents waiting without any PDD services, 70 percent had not met with a caseworker to develop an Outcome Plan — the document that drives all subsequent service decisions and budgets under the program.

For respondents already receiving PDD supports, 41 percent reported that the supports they were receiving did not meet their needs. Inclusion Alberta documented, through its direct advocacy work with families across the province, that PDD has been operating under a *critical and urgent needs* eligibility threshold, under which 100 percent of requests not deemed critical, urgent, or related to employment are being rejected. The report describes the threshold as a structure under which adults with intellectual disabilities are denied support until their circumstances deteriorate to the point of crisis, and notes that requests are frequently rejected informally by caseworkers without ever being documented or submitted to a manager. Where formal decisions are not issued, families are not informed of their right to appeal.

Inclusion Alberta's executive summary of the survey is direct:

"Respondents describe valuable years of life being squandered for adults who remain isolated at home, their skills unused, potential untapped, dreams fading, and health declining."

The program's own funding history compounds the documented harm. PDD received a budget increase in 2024 that was below the rate of population growth and inflation, in a year of unprecedented Alberta population growth that added pressure to the program. The advocacy organizations that documented these patterns lost provincial funding in March 2026, twelve months earlier than the previously communicated end date. The architecture intended to provide lifelong support to adults with developmental disabilities is, in operation, providing crisis-response triage to the subset of adults whose circumstances have deteriorated sufficiently to qualify under the urgent and critical needs threshold, while leaving the rest of the eligible population on a waitlist with no defined end.

What AISH Is Supposed to Deliver

The Assured Income for the Severely Handicapped program is the income support program for adults with severe and permanent disabilities that substantially limit the ability to earn a livelihood. As of June 2025, the program served 79,331 Albertans. The current maximum monthly living allowance is \$1,940. In addition to income, AISH provides health benefits coverage including prescription drugs, basic dental, basic optical, and certain medical equipment and supplies. The program also provides a child benefit for AISH recipients with dependent children, and personal benefits available on a case-by-case basis to recipients with no more than \$5,000 in non-exempt assets.

AISH is, in lifecycle-frame terms, a stability-mode program. The conditions that qualify a person for AISH are, by program definition, permanent. The eligibility criterion is that the disability substantially limits the ability to earn a livelihood. The program's design assumption is that recipients remain eligible across time unless their condition meaningfully changes. The architecture is, on paper, appropriate to the stability

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mode it serves.

The application process is where the structural contradiction begins. The AISH application has two parts. Part A is completed by the applicant or their personal representative and collects information on identity, age, residency, employment history, training history, income, assets, cohabiting partner status, and any dependent children. Part B is the Medical Report, completed by the applicant's physician or specialist. The Medical Report has ten sections covering applicant information, the relationship between the applicant and the completing physician, diagnoses, symptoms, levels of impairment, medication history, treatment plans, prognosis, additional supporting comments, and the physician's signature. The applicant is responsible for the cost of the physician completing the report, which can range from approximately \$100 to several hundred dollars depending on the physician and the complexity of the case. The cost is covered for applicants on Income Support; it is not covered for applicants who are not.

Once submitted, the AISH application is reviewed by a disability adjudicator who determines whether the medical eligibility threshold — severe and permanent disability that substantially limits the ability to earn a livelihood — is met. The applicant may be asked for additional information. The decision is communicated by mail. If the application is denied, the applicant has the right to appeal under the AISH appeal architecture, which historically included the Citizens Appeal Panel as an independent review body. That right has been substantially altered by the legislative changes documented later in this section.

For applicants newly turning eighteen, the application process must be initiated approximately six months before the eighteenth birthday, per Government of Alberta guidance. For applicants who already qualify for PDD, the PDD approval substitutes for the Part B Medical Report. For all other applicants, the full process — physician appointment, payment, completion, submission, adjudication, decision — must be navigated independently, in advance, by a person whose disability is the basis for the application being made.

The application is the entry point into the stability mode. It is also, by its design, a growth-mode test — it requires the applicant to demonstrate the executive function, administrative capacity, financial wherewithal, and forward planning that the population the program serves is, by program definition, substantially impaired in. The contradiction is examined in the next section.

The Structural Contradiction

The Government of Alberta has, through the AISH adjudication process, formally determined that 79,331 Albertans have severe and permanent disabilities that substantially limit their ability to earn a livelihood. The determination was made by Alberta's own contracted medical professionals, against Alberta's own statutory criteria, after review of medical evidence submitted by physicians who treat the applicants. Each individual determination is the province's own formal finding that the population in question requires the support the program provides.

The same Government of Alberta has designed an ongoing compliance and reassessment architecture that requires those same 79,331 Albertans to perform the following administrative tasks, on an ongoing basis, throughout the period of their eligibility: read and complete forms written at a Grade 12 to 16 reading level; navigate phone trees, online portals, and call-centre service models without continuity of

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caseworker relationship; produce annual income tax filings with no taxable income and no refund; respond to information requests, provide updated medical documentation, and contest pharmacy denials, billing errors, and benefit reductions when they occur; meet deadlines that do not flex for medical flares, hospitalizations, or the cognitive consequences of the conditions the program serves; and, beginning July 1, 2026, navigate the transition to a new program (ADAP) and the reassessment that transition entails, without the right to appeal the transition decision itself.

The defining clinical features of the largest AISH diagnostic categories — cognitive disorders (28.3% of the caseload), mental illness disorders (30.2%), and the cognitive-mental-health overlap that produces multi-condition profiles in many recipients — include difficulty with sustained executive function; difficulty processing complex written communication; difficulty initiating, sequencing, and completing multi-step administrative tasks; difficulty navigating institutional systems without external support; reduced reliable access to phone and digital communication during symptomatic periods; and reduced energy reserves available for the kind of administrative work the system requires. These are not aspersions cast on the population. They are the diagnostic criteria. They are what the medical professionals contracted by Alberta documented, and what the AISH adjudication process formally accepted, when the population was approved for the program in the first place.

The contradiction is structural, not individual. The system has approved the population on the basis of limitations that the system then ignores when designing the compliance architecture the population must navigate. The cost is documented: every reassessment burden, every form to refile, every appeal to mount, every pharmacy denial to contest, consumes the energy the population has available for functioning. The destabilization is the harm. The program designed to provide stability is, in operation, the source of the destabilization. The recipients describe this constantly. The system does not, in its current design, register the description as a problem with the system. It treats the description as a complaint about a process that the recipients are, in the system's view, simply required to complete.

The structural contradiction is the lifecycle frame's central diagnosis at the adulthood stage. A stability-mode program operated through growth-mode testing logic is not a stability-mode program. It is a stability-mode program in name, with growth-mode demands attached, applied to a population the system has already determined is not in the growth mode. The harm is foreseeable. The harm is foreseen. The harm is documented. The architecture continues.

The Compound Failure: Bill 12 and the AISH-to-ADAP Transition

On December 9, 2025, the Government of Alberta passed Bill 12, the *Financial Statutes Amendment Act, 2025*. The bill made fourteen separate legislative changes simultaneously, including the structural restructuring of Alberta's adult disability income support system. The changes specific to AISH and the new Alberta Disability Assistance Program (ADAP) include the following:

The AISH definition is narrowed. Under Bill 12, AISH eligibility now requires *a severe disability that permanently prevents employment*. The previous statutory criterion — *substantially limits the ability to earn a livelihood* — is replaced by a stricter binary test: prevention of employment, rather than substantial limitation of earning capacity. The detailed regulatory definition of *prevents employment* is not in the legislation. It will be set in regulation, after the legislation has passed, by the Minister.

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ADAP is created as a separate program. ADAP eligibility requires a *severe disability that substantially impedes employment*. The detailed regulatory definition of *substantially impedes* is also not in the legislation. It will also be set in regulation. The two definitions sit alongside each other in the same statute without further specification, leaving the operational distinction between *prevents* and *substantially impedes* to be determined by the Minister at a later date.

All current AISH recipients are automatically transferred to ADAP. Section 12.6 of Bill 12 grants the government the authority to reassess and reclassify existing AISH recipients into ADAP. To remain on AISH, each recipient must reapply through a new Medical Review Panel and demonstrate that they meet the narrower *prevents employment* standard. The Medical Review Panel's decisions are final.

Appeals of the AISH-to-ADAP transition decision are barred. Section 12.8 of Bill 12 specifies that when an individual is transitioned from AISH to ADAP, the transition decision cannot be appealed.

The Minister can limit other appeal categories. Section 10(2)(e) of Bill 12 grants the Minister authority to determine which categories of decisions the Citizens Appeal Panel is permitted to hear. If the Minister removes a category from appeal jurisdiction, no further independent review of decisions in that category is available.

Cost-of-living indexation is removed from legislation. Bill 12 repeals Schedule 1 of the prior statute, which had outlined the legislated mechanism for adjusting AISH benefits by the Alberta Escalator. Future cost-of-living adjustments are left to regulation and ministerial discretion, with no statutory floor.

The financial consequences for recipients moved from AISH to ADAP are documented. The maximum monthly living allowance for AISH is currently \$1,940. For ADAP, the maximum is \$1,740 — a \$200 reduction, equivalent to a 10 percent cut. The earnings exemption shrinks dramatically: under AISH, the first \$1,072 per month of employment income is fully exempt; under ADAP, the fully exempt amount is \$350 per month, with benefits then phased out on a sliding scale until eliminated entirely at approximately \$45,000 in annual income. Inclusion Alberta has documented that the earnings exemption reduction represents a 67 percent cut to the fully exempt portion.

The \$200 monthly reduction is the difference, for many recipients, between marginal subsistence and acute hardship. One AISH recipient interviewed by Inclusion Alberta described the figure directly:

"The difference between getting uncovered prescriptions paid for or having groceries for the month. The difference between almost living and barely existing."

The Government of Alberta's stated rationale for ADAP is that it expands employment supports for Albertans with disabilities who are still able to work. The data does not support the rationale. As of June 2025, of the 79,331 Albertans on AISH, approximately 13,000 (16 percent) had some employment earnings. Over 66,000 (84 percent) had no employment earnings. University of Calgary economist Gillian Petit, quoted in CTV News reporting on Bill 12, described the 84-percent figure as a *very big hurdle* for the government's work-first rationale. Inclusion Alberta has documented that even minimal work would not, under the ADAP design, leave most recipients better off than under the current AISH structure unless they could secure employment income exceeding \$350 per month at sustainable rates — a threshold that is structurally unrealistic for the population the program serves, in a labour market with provincial

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unemployment exceeding 8 percent and youth unemployment exceeding 16 percent in 2025.

Five Alberta municipalities — Red Deer, Edmonton, Lethbridge, Camrose, and Calgary — have passed or initiated motions calling on the province to pause the ADAP transition. Bradley LaFortune of Public Interest Alberta has stated publicly that changing fourteen laws at once and stripping indexation and appeal rights undermines transparency and access to administrative fairness. The Minister of Assisted Living and Social Services has responded that the criticism is misinformation and that the transition will proceed as scheduled.

The transition takes effect July 1, 2026.

BILL 12 — KEY STRUCTURAL CHANGES TO AISH

Bill 12 narrows AISH eligibility — from *substantially limits the ability to earn a livelihood to permanently prevents employment*. The detailed definition is left to regulation.

All 79,331 current AISH recipients are automatically transferred to ADAP on July 1, 2026. Section 12.6 grants the authority. To remain on AISH, recipients must reapply through a new Medical Review Panel whose decisions are final.

Appeal of the transition decision is barred under Section 12.8. The Minister gains authority under Section 10(2)(e) to limit which categories of other decisions the Citizens Appeal Panel may hear.

Statutory cost-of-living indexation is removed. Schedule 1 of the prior statute, which legislated annual adjustment by the Alberta Escalator, is repealed. Future adjustments are at regulatory and ministerial discretion.

The financial cut is documented. AISH maximum: \$1,940/month. ADAP maximum: \$1,740/month. The \$200 monthly reduction equals 10 percent. Earnings exemption cut from \$1,072 to \$350 (a 67 percent reduction in the fully exempt portion).

The data does not support the rationale. 84 percent of AISH recipients have no employment earnings. Provincial unemployment exceeds 8 percent; youth unemployment exceeds 16 percent. Five municipalities — Red Deer, Edmonton, Lethbridge, Camrose, Calgary — have called for a pause.

What the Lifecycle Failure Produces

The PDD waitlist, the AISH adjudication burden, and the AISH-to-ADAP transition together constitute the adult-stage manifestation of the lifecycle failure pattern documented across Part One. The pattern is consistent at each stage. A program designed to deliver one mode of support — growth in childhood under FSCD, lifelong support in adulthood under PDD, stability in adulthood under AISH — is operated

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under a different mode's logic, with predictable, documented harm to the population the program was designed to serve.

For the disabled child whose FSCD eligibility ends at eighteen, the cliff is the entry point. The child becomes an adult who must independently navigate two separate program applications, secure physician appointments and pay physician fees, document medical history that has been compiled over a lifetime, and meet administrative deadlines that the disability the application is for routinely impairs the capacity to meet. If PDD applies, the wait for an eligibility decision averages four and a half months, with thirty-nine percent waiting ten months or longer. The wait for services after a positive decision averages fourteen months. If AISH applies, the application can be initiated six months before the eighteenth birthday but the adjudication process operates on its own timeline. The newly-adult disabled Albertan who turned eighteen yesterday is, in many cases, navigating these processes from a position of no income, no caseworker, no continuity of childhood support, and no established adult medical relationships of their own.

For the family that has supported the child through childhood, the cliff is the moment at which the institutional knowledge they built about FSCD becomes obsolete and the institutional knowledge they need about PDD and AISH does not yet exist. The advocacy organizations that translated community knowledge into navigation support — Inclusion Alberta, the Disability Action Hall, the affiliates of both — have been progressively defunded, as documented in Sections 1.1 and 1.3. The family is, in many cases, navigating the transition without external support, while continuing to provide the care the adult systems are slow to absorb.

For the system, the cliff is invisible by design. The PDD program tracks the urgent and critical cases it approves; it does not, per Inclusion Alberta's documentation, accurately track the population that is denied or that does not receive a formal decision. The AISH program tracks the recipients who navigate the application successfully; it does not track the disabled adults who never apply because the application barrier is insurmountable, or who apply, are denied, and do not appeal because they were not informed of the right to appeal. The compound failure is borne by the population. The aggregate cost is borne by every downstream system — healthcare, housing, justice, family caregivers, emergency services — that absorbs the consequences of the upstream program failures.

The lifecycle failure pattern is not a series of separate program problems. It is one coordinated structural failure that recurs at each life stage. Section 1.3 documented the failure in childhood. Section 1.4 has documented it at the transition to adulthood. Section 1.5 examines what happens when the family caregivers who absorb the system's failures themselves fail — through illness, exhaustion, or death — and the disabled adult is left without the wraparound support the public architecture was supposed to have built.

What the Adulthood Threshold Looks Like

All figures sourced from Government of Alberta open data, Statistics Canada, federal statute, UN treaty, and peer-reviewed research.

ILLUSTRATION — A FAMILY AT THE EIGHTEENTH BIRTHDAY

Imagine a family at the beginning of the calendar year their child turns eighteen. The child has been on the FSCD waitlist for parts of the previous decade. The child has received some services, some of the time. The child is autistic with a co-occurring intellectual disability and a co-occurring anxiety disorder. The child is, by Alberta's own clinical thresholds, eligible for PDD. The child is also, by the same thresholds, eligible for AISH on completion of the application.

The family attempts to apply for AISH six months before the child's eighteenth birthday, per the Government of Alberta's guidance. The family doctor of fifteen years has retired and not been replaced; the child has been on a wait list for a new family physician for two years. The walk-in clinic the family has used for acute care will not complete the AISH Part B Medical Report, which requires a longitudinal clinical relationship the walk-in clinic does not have. The family contacts a private psychiatric assessment service that will complete the Medical Report. The fee is \$700. The family pays it.

The family also applies to PDD. The PDD application requires a Psychological Assessment Report from a registered psychologist. The wait for a publicly funded assessment is approximately eighteen months. The family pays \$2,400 privately to obtain the assessment in eight weeks. The PDD application is submitted. The family waits for an eligibility decision. The average wait, per Inclusion Alberta data, is four and a half months. After an eligibility decision is received, the average wait for services is an additional fourteen months. The child will be nineteen and a half years old before PDD services begin. The child is eighteen now.

The AISH application is processed. The child is approved. The first AISH payment arrives in the month after the eighteenth birthday at the maximum monthly living allowance of \$1,940. The family is grateful. The family also notes that on July 1, 2026, the child will be transferred automatically to ADAP. The new monthly amount will be \$1,740. The family does not yet know whether the child will be among those whom the new Medical Review Panel determines to be permanently unable to work, in which case they will be returned to AISH at \$1,940. They do know that if the Medical Review Panel does not so determine, the child will remain at \$1,740, and the transition decision cannot be appealed.

The family does the math. The PDD waitlist will continue for another fourteen months after the eligibility decision arrives. The AISH-to-ADAP transition is in five months. The Canada Disability Benefit, which would otherwise have provided \$200 per month in federal support, is being clawed back dollar-for-dollar by the Alberta government, producing no net increase. The advocacy organizations that would have helped the family navigate any of this have lost provincial funding. The family is on its own.

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The child does not know any of this, in the same way the child did not know about the FSCD wait at age two. What the child knows is that the routines of high school have ended, the structure that organized the previous twelve years has ended, and the adult systems the family has been describing for the past three years are, in operation, mostly waiting rooms.

What This Section Establishes

The transition from childhood to adulthood is the second documented failure point in the lifecycle frame. PDD operates with multi-year waitlists and a *critical and urgent needs* threshold that, per Inclusion Alberta's documentation, denies one hundred percent of requests not deemed critical, urgent, or related to employment. AISH operates with an application architecture that requires, for entry, the precise administrative capacities the disability the application tests for routinely impairs. The Bill 12 restructuring of the adult income support architecture — passed December 9, 2025, taking effect July 1, 2026 — narrows AISH eligibility, transfers all current recipients to a lower-benefit replacement program, removes statutory cost-of-living indexation, bars appeal of the transition decision, and grants the Minister discretion to limit appeal jurisdiction across the program architecture more broadly.

The blueprint Part Three builds will require, at minimum, the following commitments at the adult-transition level: integrated transition planning beginning at age sixteen with the FSCD and adult-systems caseworkers operating jointly; maximum statutory wait times for both PDD eligibility decisions and PDD service commencement; restoration of the Citizens Appeal Panel's full independent jurisdiction across both AISH and ADAP; legislated cost-of-living indexation protected from regulatory rollback; mandatory provision of adult-system entry support for transitioning eighteen-year-olds, with no requirement that the family pay privately for assessments the public system is structurally incapable of providing within the relevant window; and a binding statutory recognition that the disability the program serves cannot be operationalized as a barrier to navigating the program. The legislation Part Three proposes is the architecture that makes those commitments enforceable. The argument Part Four makes is that the Bill 12 direction is a choice, made deliberately, with full knowledge that it imposes the heaviest administrative burden on the population least able to absorb it.

Section 1.5 follows the population from Section 1.4 into the next life stage: the family caregivers who currently absorb what the public systems do not provide, the documented caregiver burden that compounds across decades, and what happens when the primary caregiver, who has been the practical infrastructure of the disabled adult's life, ages, weakens, or dies. The lifecycle frame's fourth failure point is the moment the family safety net the province has tacitly relied on is no longer available. The province has not built the architecture that the family has been substituting for. Section 1.5 examines what that means for the population the architecture was supposed to serve.